

Psoriasis

A Chronic Manifestation

AN INTRODUCTION

Today, 2-3% of the world's population is affected by Psoriasis, which is an immune-mediated, proliferative and inflammatory disease of the skin, which mainly affects people who are genetically predisposed. It is a chronic disease characterized by variants that include plaque, flexural and guttate psoriasis which may manifest itself in variety of forms at any age, from infancy to old age, having an equal distribution in both sexes. Several studies have shown a bimodal curve for the age of onset, with the mean age of onset for the first peak ranging from 15-20 years, and the second peak occurring at 55-60 years of age.

INDIAN SCENARIO

The incidence of psoriasis in India ranges from 1.4% to 2.3% of all dermatology out patients. Family history ranges from 2% to 13.6%. Plaque type disease is the commonest with the scalp and extremities being the common sites involved. Nail changes are seen in more than 70% in certain studies. Joint involvement, however, is rare around 4%. Psychiatric morbidity is found in more than half of the patients suffering from psoriasis. Paediatric cases constitute 5-10% of all psoriatic patients. The average age of onset in paediatric population is 7 years.

CAUSES BEHIND PSORIASIS

Several researches have been done to find the causes of Psoriasis, but no root cause has yet been discovered. Psoriasis has nothing to do with poor skin care or hygiene. It is not known exactly what causes psoriasis, but it is believed to have a genetic component. Most researchers agree that the immune system is triggered, which speeds up the growth cycle of skin cells. A normal skin cell matures and sheds from the body's surface in 28 to 30 days. But a psoriatic skin cell takes only five to six days to mature and move to the surface. Instead of falling off (shedding), the cells pile up and form the lesions or silvery scales.

Triggers can include emotional stress, injury to the skin, some types of infection and reactions to certain drugs. Stress can cause psoriasis to flare up for the first time or aggravate existing psoriasis. Psoriasis can also be triggered in areas of the skin that have been injured or traumatized. This is known as the "Koebner phenomenon". Vaccinations, sunburns and scratches can all trigger a Koebner response. Certain medications, like antimalarial drugs, lithium and certain beta-blockers, are also known to cause psoriasis to flare up.

Psoriasis is a hereditary condition. This does not mean that psoriasis is automatically passed on to the children but if one of the parents has psoriasis, there is a 15-25% increased risk. If both parents are suffering from psoriasis, the risk increases to 70%. Unfortunately, psoriasis is chronic and tends to flare up throughout life. There is no cure but a range of products exist which treat psoriasis and relieves the symptoms.

PSORIATIC VARIANTS

Plaque Psoriasis is the most common form of psoriasis. It affects 80-90 percent of the psoriasis affected persons. Plaques typically appear as raised areas of inflamed skin covered with silvery scales. Extensor surfaces like elbows, hands, trunk and knees are commonly involved.

Flexural Psoriasis appears as smooth inflamed patches of skin. It occurs in skin folds, particularly around genitals

(between the thigh and groin), armpits and under the breasts. It is aggravated by friction and sweat and is vulnerable to fungal infections.

Guttate Psoriasis is characterized by numerous small oval spots like raindrops. These spots of psoriasis appear over large areas of the body, commonly involving the trunk. It is associated with streptococcal throat infection.

Pustular psoriasis can be localized or generalized with widespread patches occurring randomly on any part of the body.

Nail psoriasis produces a variety of changes in the appearance of finger and toenails. These changes include discolouring under the nail plate, pitting of nails, lines going across the nails, thickening of the skin under the nail, and the loosening (onycholysis) and crumbling of the nail.

Erythrodermic psoriasis involves the widespread inflammation of the skin over most of the body surface. It may be accompanied by severe itching, inflammation and pain. It is often the result of an exacerbation of unstable plaque psoriasis, particularly following the abrupt withdrawal of systemic treatment. This form of psoriasis can be fatal, as the extreme inflammation disrupts the body's ability to regulate temperature and for the skin to perform barrier functions.

Psoriatic arthritis involves joint along with skin involvement. Psoriatic arthritis can affect any joint but is most common in the joints of the fingers and toes. This can result in swelling of fingers and toes and can also affect the hip joints, knees and the spine.

Diagnosis of psoriasis is usually based on the appearance of the skin. There are no special blood tests or diagnostic procedures for psoriasis. Sometimes a skin biopsy or scraping may be needed to rule out other disorders.

PSORIASIS AND QUALITY OF LIFE

Psoriasis has a major negative impact on a patient's quality of life. Health related factors affecting the quality of life include physical, mental, functional and emotional factors. Also it can affect non-health related elements such as job, family, friends and compared to healthy adults, the social functioning of psoriasis patients is markedly reduced as psoriasis is often misconstrued as contagious.

These misconceptions deprive psoriasis patients of public facilities. Among patients with psoriasis, many experience problems with receiving equal service or treatment in various service establishments, such as hair salons, barbershops, public pools, health clubs etc.

PSORIASIS TREATMENT OPTIONS

The choice of treatment depends on the type of psoriasis, percentage of body surface area involvement (BSA) and the patient who needs to be treated. The line of treatment provided to the patients therefore could be either topical (application on the skin), systemic or phototherapy. The key to success for topical therapy is compliance, i.e. applying the right amount of cream/ointment as required. Patients often try various treatments before finding the right one that works for them.

Topical therapy (creams and ointments) is normally the first line of treatment. It includes vitamin D analogues, salicylic acid, dithranol, corticosteroids and retinoids.

Vitamin D analogues are used for mild to moderate psoriasis. The new combination product Daivobet®/ Dovobet® combines the benefits of a vitamin D analogue: calcipotriol and a corticosteroid (betametasone dipropionate) and is effective in chronic plaque psoriasis with BSA<30% involvement. Daivobet®/ Dovobet® is the only product of its kind and distinguishes itself by a once daily application providing fast onset of action and a rapid relief of symptoms while maintaining a superior safety profile.